



Warning and Precautions

Although there is no evidence of rebound seizures with gabapentin, abrupt withdrawal of anticonvulsants in epileptic patients may precipitate status epilepticus.

Gabapentin is not generally considered effective in the treatment of absence seizures.

Patients who require concomitant treatment with morphine may experience increases in gabapentin concentrations. Patients should be carefully observed for signs of CNS depression, such as somnolence, and the doses of gabapentin or morphine should be reduced appropriately.

Respiratory depression

Gabapentin has been associated with severe respiratory depression. Patients with compromised respiratory function, respiratory or neurological disease, renal impairment, concomitant use of central nervous system (CNS) depressants and the elderly might be at higher risk of experiencing this severe adverse reaction. Dose adjustments might be necessary in these patients.

Information for Patients:

To assure safe and effective use of gabapentin, the following information and instructions should be given to patients:

- 1. Inform the physician about any prescription or nonprescription medications, alcohol or drug the patient is currently taking or plan to take during treatment with gabapentin.
- 2. You should inform your physician if you are pregnant, or if you are planning to become pregnant, or if you become pregnant while you are taking gabapentin.
- 3. Gabapentin is excreted in human milk, and the effect on the nursing infant is unknown. You should inform your physician if you are breast feeding an infant.
- 4. Gabapentin may impair your ability to drive a car or operate potentially dangerous machinery. Until it is known that this medication does not affect your ability to engage in these activities, do not drive a car or operate potentially dangerous machinery.
- 5. You should not allow more than 12 hours between gabapentin doses to prevent breakthrough convulsions.

Interaction with other Medicaments

No interaction between gabapentin and phenobarbital, phenytoin, valproic acid, or carbamazepine has been observed.

Gabapentin steady-state pharmacokinetics are similar for healthy subjects and patients with epilepsy receiving these antiepileptic agents.

Coadministration of gabapentin with oral contraceptives containing norethindrone and/or ethinyl estradiol, does not influence the steady-state pharmacokinetics of either component.

Co administration of gabapentin with antacids containing aluminium and magnesium, reduces gabapentin bioavailability up to 20%. It is recommended that gabapentin be taken at the earliest two hours following antacid administration.

Renal excretion of gabapentin is unaltered by probenecid.

A slight decrease in renal excretion of gabapentin that is observed when it is coadministered with cimetidine is not expected to be of clinical importance.

Pregnancy and Lactation

There are no adequate data from the use of gabapentin in pregnant women.

The potential risk for humans is unknown. Gabapentin should not be used during pregnancy unless the potential benefit to the mother clearly outweighs the potential risk to the fetus.

Gabapentin is excreted in human milk. Because the effect on the breast-fed infant is unknown, caution should be exercised when gabapentin is administered to a breastfeeding mother. Gabapentin should be used in breast-feeding mothers only if the benefits clearly outweigh the risks.

Effects on ability to drive and use machines:

Gabapentin may impair the patient's ability to drive a car or operate potentially dangerous machinery. Patients should be advised not to drive a car or operate potentially dangerous machinery. Until it is known that Neurontin does not affect their ability to engage in these activities, do not drive a car or operate potentially dangerous machinery.

Side Effects

The adverse reactions observed during clinical studies conducted in epilepsy adjunctive Therapy frequently occurring treatment-emergent signs and symptoms during placebo controlled studies are summarized as follows:

Body as a Whole: Asthenia, malaise, facial edema.

Cardiovascular System: Hypertension.

Digestive System: Flatulence, anorexia, gingivitis.

Hemic/Lymphatic Systems: Purpura most often described as bruises resulting from physical trauma.

Musculoskeletal System: Arthralgia.

Nervous System: Vertigo; hyperkinesia; increased, decreased or absent reflexes; paresthesia, anxiety, hostility.

Respiratory System: Pneumonia. Respiratory, thoracic and mediastinal disorders

Rare: Respiratory Depression

Urogenital System: Urinary tract infection.

Special Senses: Abnormal vision most often described as a visual disturbance.

Post-marketing experience: Dysphagia

Children: The most commonly observed adverse events reported with the use of gabapentin in combination with other antiepileptic drugs in children 3-12 years, not seen in equal frequency among placebo-treated patients, were viral infection, fever, nausea and/or vomiting, and somnolence.

Symptoms and Treatment of Overdose

Acute, life-threatening toxicity may not occur with gabapentin overdoses of up to 49 g.

Symptoms of the overdoses included dizziness, double vision, slurred speech, drowsiness, lethargy and mild diarrhoea. All patients recovered fully with supportive care. Reduced absorption of gabapentin at higher doses may limit drug absorption at the time of overdosing and, hence, minimize toxicity from overdoses.

Although gabapentin can be removed by haemodialysis, based on prior experience it is usually not required. However, in patients with severe renal impairment, haemodialysis may be indicated.

Storage Condition

Do not store above 30 °C.

Shelf Life

24 months

Presentation

Ten Tablets are packed in triple laminated cold form -Aluminium foil blister pack; three such blisters are packed in a printed Carton along with a package insert.
Pack size: 3 X 10's

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Manufactured by:
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